

Discharge Summary

Patient: Armin Schulze
Date of Birth: 08/13/1960
Admission Date: 10/13/2025
Discharge Date: 10/21/2025
Attending Physician: Dr. Sabine Neumann, MD

Mr. Armin Schulze was admitted for optimization of diabetes therapy in the context of known retinopathy.

Primary Diagnosis: Type 2 diabetes mellitus with retinopathy (E11.3)

Laboratory values:

Parameter	Value	Reference
HbA1c	9.1%	< 7.5%
Creatinine	1.3 mg/dL	0.7–1.2 mg/dL
Glucose	224 mg/dL	70–100 mg/dL

Target HbA1c < 7.5% discussed. Discharged 10/21/2025.

Relevant Medical History:

The patient has a long-standing history of arterial hypertension diagnosed in 2008, currently managed with an ACE inhibitor and a calcium channel blocker. Type 2 diabetes mellitus was diagnosed in 2015 and is managed with oral antidiabetic agents and insulin. Additionally, the patient has a history of hypercholesterolemia and is on statin therapy. There is no known history of malignancy, significant surgical procedures, or major allergies.

Family history is notable for coronary artery disease in the patient's father, who suffered a myocardial infarction at age 62. The patient is a former smoker with a 20 pack-year history, having ceased smoking in 2010. Alcohol consumption is reported as moderate.

Follow-up Recommendations:

We recommend the following follow-up measures: (1) General practitioner visit within 2 weeks for medication review and blood pressure control. (2) Laboratory workup including complete blood count, renal function, liver enzymes, and HbA1c in 3 months. (3) Specialist referral as outlined above. (4) Repeat imaging in 6 months if clinically indicated.

The patient has been fully informed of the diagnosis, treatment plan, and recommended follow-up schedule. Written discharge instructions were provided. The patient expressed understanding and willingness to comply with the recommended measures.

Medication on Discharge:

The following medications were prescribed at discharge: metformin 1000 mg twice daily, ramipril 5 mg once daily, atorvastatin 40 mg once daily, aspirin 100 mg once daily, and pantoprazole 40 mg once daily. The patient was counseled on the importance of adherence and potential side effects.

Dr. Sabine Neumann, MD
Senior Physician, Diabetology